INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Anastrozole · daily hormone therapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Regimen source
NCCP 00254
Version 5

Continuous daily tablets

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

After early breast cancer treatment, endocrine therapy reduces the risk of recurrence. In advanced disease, the aim is to control cancer and symptoms. Your setting determines the duration and follow-up.

HORMONE THERAPY · AROMATASE INHIBITOR

Anastrozole

Reduces the oestrogen available to stimulate cancer cells

Aromatase normally makes oestrogen → The medicine blocks aromatase → Less oestrogen reaches cancer-cell receptors

Anastrozole blocks aromatase, an enzyme that makes oestrogen in fat and other tissues after menopause. Less oestrogen is available to stimulate hormone-sensitive breast cancer cells. This also explains effects such as hot flushes, vaginal dryness and bone thinning. It does not reliably switch off functioning ovaries; any ovarian-suppression treatment needs a separate prescription and checks.

Tell the team if joint pain, hot flushes, vaginal dryness or sleep disturbance affects daily life. Bone thinning may cause no early symptoms, so keep the agreed bone-health reviews.

The treatment being discussed

Who this information covers

Postmenopausal hormone-sensitive breast cancer in the NCCP adjuvant, switch-after-tamoxifen or advanced settings. The exact receptor and prior-treatment criteria remain a prescribing decision.

NCCP 00254 v5 · official regimen

Medicine and schedule

Take anastrozole once daily continuously. There is no planned week off. The treatment setting determines the intended duration. Swallow the tablet whole with water, with or without food. Do not replace a missed or vomited dose; resume the next scheduled dose.

NCCP 00254 v5 · official regimen

Review and duration

Swallow the tablet whole with water, with or without food. Do not replace a missed or vomited dose; resume the next scheduled dose. A change from tamoxifen does not automatically restart the whole treatment clock. Ask which earlier treatment counts towards your plan.

NCCP 00254 v5 · official regimen

Individual risks and support

A functioning premenopausal ovarian state is not compatible with aromatase inhibitor (AI) monotherapy. Severe hepatic impairment is not recommended by NCCP; moderate impairment requires caution. Confirm menopausal status rather than assuming that absent periods after chemotherapy mean permanent menopause. Any ovarian-suppression treatment must have its own prescribed schedule.

NCCP 00254 v5 · official regimen

Why this endocrine treatment is offered

For hormone-sensitive early breast cancer, endocrine treatment helps reduce the chance of the cancer returning. For advanced disease, it can help control cancer and symptoms.

Your likely benefit depends on your cancer, previous treatment and health. The treating team explains why this option fits your plan.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Report persistent limiting joint pain, new tendon pain or swelling, a possible fracture, jaundice or new vaginal bleeding. Sudden loss of tendon function needs prompt assessment. Severe allergic symptoms, chest pain or major breathing difficulty require urgent help.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Read the row for your treatment setting. Columns show order, not equal lengths of time. Agree the due window, booking team, result reviewer and whom to contact if a check is missing.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00254 v5 · official regimen · HPRA Anastrozole SmPC cited by NCCP · Pan-Asian adapted ESMO early breast cancer guidelines · Pan-Asian adapted ESMO metastatic breast cancer guidelines · NICE NG 101: early and locally advanced breast cancer · ESO-ESMO ABC 5 consensus: tumour-marker interpretation · IOF/CABS/ECTS/IEG/ESCEO/IMS/SIOG updated joint statement on aromatase-inhibitor-associated bone loss

THE WHOLE TREATMENT JOURNEY

Anastrozole · daily hormone therapy: treatment and checks

Your treatment pattern

Continuous daily tablets

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

Postmenopausal hormone-sensitive breast cancer

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First weeks	During treatment	Longer-term review	Change / completion
Treatment plan NCCP 00254	Confirm indication and prescription	Start the agreed daily / injection schedule	Continue unless prescriber changes the plan	Review duration, benefit and tolerability	Agree continuation, switch or completion
Bloods / symptom review NCCP 00254	full blood count (FBC), renal and liver profile	Review tolerability and medicines	Further tests when clinically indicated	Review symptoms, adherence and results	Follow unresolved abnormalities
electrocardiogram (ECG) / echo NCCP 00254	No routine ECG / echo for this medicine alone	Investigate new cardiac symptoms	Further cardiac tests by risk / symptoms	Companion-drug cardiac rules remain separate	Follow abnormalities when indicated

Care / check	Before starting	First weeks	During treatment	Longer-term review	Change / completion
Bone density / dual-energy X-ray absorptiometry (bone density) (DXA) NCCP 00254 · AIBL2025	Adjuvant AI: baseline DXA unless already on adjuvant bisphosphonates	Review previous DXA and fracture risk	No cycle-based DXA appointments	Lower-risk AI: reassess in 1–2 years; individualise	Continue bone care according to risk
Early-breast surveillance NCCP 00254	Confirm early / curative setting	Set first follow-up mammogram date	Assess new symptoms promptly	Annual mammography of remaining breast tissue	No routine computed tomography (CT) / markers if asymptomatic
Advanced-disease response NCCP 00254	Document disease sites / baseline assessment	New symptoms can bring assessment forward	Generally every 2–4 months; individualise	Adapt interval and imaging to disease / benefit	Markers supplement, never replace, assessment
Early: radiotherapy ESMO_EBC2024	Review surgery / pathology	Plan radiotherapy if indicated	Coordinate endocrine timing	Follow the early-breast plan	Review late effects when relevant

What each check is for

1 Confirm the plan

Before starting or changing treatment

Postmenopausal hormone-sensitive breast cancer in the NCCP adjuvant, switch-after-tamoxifen or advanced settings. The exact receptor and prior-treatment criteria remain a prescribing decision.

Ask / act: Confirm treatment setting, medicines, menopause and the planned review.

2 Checks and symptoms

At the agreed review points

FBC, renal and liver profile. Confirm menopause; hormone tests may be needed if it is uncertain. Assess fracture risk and baseline bone density, particularly for adjuvant AI treatment. The NICE adjuvant baseline-DXA recommendation has an exception for patients already receiving adjuvant bisphosphonates. Assess symptoms, adherence and bone health at reviews. Repeat lipids and other blood tests when clinically indicated. Bone-density timing follows baseline results and fracture risk.

Ask / act: Agree who books tests, reviews results and responds to new symptoms.

3

Review ongoing benefit

At setting-specific decision points

After early breast cancer treatment, endocrine therapy reduces the risk of recurrence. In advanced disease, the aim is to control cancer and symptoms. Your setting determines the duration and follow-up.

Ask / **act:** Review duration, alternatives and patient priorities; do not stop or extend treatment automatically.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Checks throughout treatment

Your team arranges tests and explains the results. A planned check is not confirmation that it has been completed.

Treatment-specific checks

FBC, renal and liver profile. Confirm menopause; hormone tests may be needed if it is uncertain. Assess fracture risk and baseline bone density, particularly for adjuvant AI treatment. The NICE adjuvant baseline-DXA recommendation has an exception for patients already receiving adjuvant bisphosphonates. Assess symptoms, adherence and bone health at reviews. Repeat lipids and other blood tests when clinically indicated. Bone-density timing follows baseline results and fracture risk.

NCCP 00254 v5 · official regimen

Heart checks

ECG or other heart tests depend on symptoms and personal risk; this medicine alone does not require a routine HER2-style echocardiogram schedule.

NCCP 00254 v5 · official regimen

Bone health

Offer baseline DXA when starting an adjuvant aromatase inhibitor if not already receiving adjuvant bisphosphonates; assess fracture risk and relevant previous results. Under the 2025 joint guidance, patients with a T-score above –2.0 and no additional risk factors generally have risk and bone density reassessed every one to two years. Patients on bone-protective treatment generally have BMD reassessed about every two years. Fracture, substantial bone loss or a change in risk can require earlier review. The clinician selects the interval and any bone treatment; these are not automatic yearly scans for everyone.

NICE NG 101: early and locally advanced breast cancer · IOF/CABS/ECTS/IEG/ESCEO/IMS/SIOG updated joint statement on aromatase-inhibitor-associated bone loss

Your setting changes the pathway

Choose the pathway that your team has confirmed; these are not all extra treatments you must have.

Early-breast follow-up

After curative treatment, annual mammography is generally used for remaining breast tissue: both breasts after conservation or the opposite breast after unilateral mastectomy. The mastectomy side is not routinely mammographed. Routine CT, bone scans and tumour markers are not recommended for asymptomatic early-breast surveillance. New symptoms need assessment between appointments.

Pan-Asian adapted ESMO early breast cancer guidelines · NICE NG 101: early and locally advanced breast cancer

Surgery, chemotherapy and radiotherapy

In an early-breast pathway, chemotherapy, if indicated, precedes endocrine treatment. Surgery and its pathology guide adjuvant choices. Postoperative radiotherapy is usually delivered after recovery and any adjuvant chemotherapy; aromatase inhibitors may overlap radiotherapy under the agreed plan. Tamoxifen timing around radiotherapy is agreed by the treating teams. None of these additional treatments is automatically required by an endocrine prescription.

Pan-Asian adapted ESMO early breast cancer guidelines · eviQ 3650 v 6: adjuvant whole-breast radiotherapy



Advanced-disease scans

For advanced breast cancer, response assessment is generally every two to four months, adapted to disease pace, sites, symptoms and treatment. Indolent disease may be reviewed less often; new symptoms can prompt earlier scans. The team chooses the imaging method; this is not a universal three-month CT chest/abdomen/pelvis order.

Tumour markers

Tumour markers may supplement imaging and clinical assessment in advanced disease if they are informative, especially if elevated at baseline. They do not replace scans, and a marker change alone should not determine treatment failure. There is no universal fixed marker timetable.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Your endocrine treatment and review milestones

These stages show the order of care, not equal lengths of time. No dates or personal details are needed. Follow the prescribed tablet schedule between reviews; the treatment setting determines when it is reviewed or completed.

Treatment or check	Timing in this view
Daily medicine	**First weeks → During treatment → Longer-term review → Change / completion** 1 mg orally once daily · continuous unless changed by prescriber
Before the first dose	**Before starting** Confirm menopause, treatment setting, baseline bloods, medicines and bone-health plan
Symptoms and tolerability	**First weeks → During treatment → Longer-term review → Change / completion** May persist or change; report troublesome symptoms early
Planned reviews	**During treatment → Longer-term review → Change / completion** Review symptoms, adherence, bone health and the planned duration or disease response

Swallow the tablet whole with water, with or without food. Do not replace a missed or vomited dose; resume the next scheduled dose.

A change from tamoxifen does not automatically restart the whole treatment clock. Ask which earlier treatment counts towards your plan.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

Practical support

Symptoms and daily life

Support medicines

No routine antiemetic, steroid premedication or granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) is required by this endocrine monotherapy. Ask for individual symptom support.

Tests and appointments

Ask who books each check, when it is due and who reviews it. Tell the team if an expected appointment has not arrived.

Taking tablets / attending injections

Practical treatment details

> **Support joint symptoms**
>
> Discuss safe physical activity, strength work and symptom treatment.

Questions to ask

When are bloods, ECGs or echoes needed?

FBC, renal and liver profile. Confirm menopause; hormone tests may be needed if it is uncertain. Assess fracture risk and baseline bone density, particularly for adjuvant AI treatment. The NICE adjuvant baseline-DXA recommendation has an exception for patients already receiving adjuvant bisphosphonates. Assess symptoms, adherence and bone health at reviews. Repeat lipids and other blood tests when clinically indicated. Bone-density timing follows baseline results and fracture risk. There is no routine ECG or echo schedule for anastrozole alone. Review cardiovascular risk factors and investigate cardiac symptoms when indicated.

Which scans apply to me?

After curative treatment, annual mammography is generally used for remaining breast tissue: both breasts after conservation or the opposite breast after unilateral mastectomy. The mastectomy side is not routinely mammographed. Routine CT, bone scans and tumour markers are not recommended for asymptomatic early-breast surveillance. New symptoms need assessment between appointments. For advanced breast cancer, response assessment is generally every two to four months, adapted to disease pace, sites, symptoms and treatment. Indolent disease may be reviewed less often; new symptoms can prompt earlier scans. The team chooses the imaging method; this is not a universal three-month CT chest/abdomen/pelvis order.

How long will treatment continue?

After early breast cancer treatment, the team sets the intended total endocrine treatment length, taking account of any earlier tamoxifen. A course of several years may later be reviewed for extension according to recurrence risk, tolerance and bone health. Advanced-disease treatment is reviewed according to benefit and tolerance. A review date is not an automatic stop date.

Do I need a port or home pump?

No port or home infusion pump is needed for this medicine itself. Any separately prescribed medicine has its own access requirements.

Can I use HRT, vaginal oestrogen or supplements?

Check with the oncology team before starting any hormone-containing treatment, including vaginal oestrogen, or any new medicine or supplement. Some can oppose this treatment or change how it works. The team can suggest symptom treatments and decide whether specialist review is needed.

What if periods return or menopause is uncertain?

Tell the team promptly about a return of periods or new vaginal bleeding. No periods after chemotherapy does not always mean the ovaries have permanently stopped working. Hormone blood tests may be needed. Ovarian suppression, if prescribed, has its own schedule and monitoring.

What should I do if symptoms make tablets difficult to continue?

Tell the team early about joint pain, hot flushes, sleep, vaginal or sexual symptoms, fatigue or missed tablets. Support, a supervised break or a change of medicine may be possible. Agree the plan with the prescriber rather than changing or stopping treatment yourself.

Do I need calcium and vitamin D?

The NCCP regimen advises daily calcium and vitamin D supplementation. Ask the team or pharmacist to confirm the suitable product and dose, taking account of your diet, blood results, kidney health and any supplements you already take. Bone-protective medicines and bone-density checks follow your individual fracture-risk plan.

Explain the plan back: What is your treatment schedule, which checks apply to your setting, and who books and reviews them?

Take anastrozole once daily continuously. There is no planned week off. The treatment setting determines the intended duration.

> Clinician supportive-care discussion draft. This is not an issued prescription.

Minimal emetogenic risk

No routine prophylactic antiemetic, corticosteroid premedication or granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) order is populated for this endocrine monotherapy. Review persistent nausea or other symptoms and prescribe individually if needed.

Medicine-specific support

- Tell the team if joint pain, hot flushes, vaginal dryness or sleep disturbance affects daily life. Bone thinning may cause no early symptoms, so keep the agreed bone-health reviews.
- No routine antiemetic, steroid premedication or G-CSF is required by this endocrine monotherapy. Ask for individual symptom support.
- Ask who books each check, when it is due and who reviews it. Tell the team if an expected appointment has not arrived.
- A functioning premenopausal ovarian state is not compatible with aromatase inhibitor (AI) monotherapy. Severe hepatic impairment is not recommended by NCCP; moderate impairment requires caution. Confirm menopausal status rather than assuming that absent periods after chemotherapy mean permanent menopause. Any ovarian-suppression treatment must have its own prescribed schedule.
- NCCP no renal adjustment including haemodialysis; severe renal impairment requires caution. No mild/moderate hepatic reduction; linked SmPC cautions moderate/severe hepatic dysfunction. NCCP severe hepatic disease not recommended: withhold pending review rather than invent a reduction.

Individual plan

Prescriber / date: _____ Pharmacy review: _____

Verify allergies, interactions, relevant organ function, treatment setting and the exact medicine before prescribing.

NCCP 00254 v5 · Monitoring and treatment map

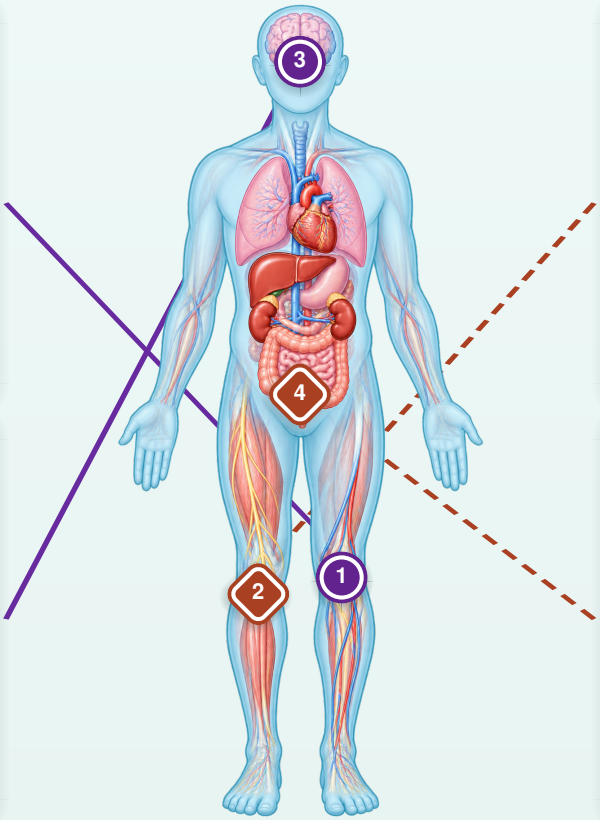
Anastrozole · daily hormone therapy

Take anastrozole once daily continuously. There is no planned week off. The treatment setting determines the intended duration.

Hormone therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Joints and muscles

Anastrozole

Pain or stiffness can affect activity; report persistent or limiting symptoms.

2 Bone strength

Anastrozole

Bone loss may be silent. Your risk assessment and bone-density plan matter.

3 Hot flushes / sleep

Anastrozole

Warm flushes and disrupted sleep can be treated; tell the team.

4 Vaginal / sexual symptoms

Anastrozole

Dryness or discomfort deserves support. Ask before using hormone-containing treatments.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Report persistent limiting joint pain, new tendon pain or swelling, a possible fracture, jaundice or new vaginal bleeding. Sudden loss of tendon function needs prompt assessment. Severe allergic symptoms, chest pain or major breathing difficulty require urgent help. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00254 v5 · official regimen**
   https://healthservice.hse.ie/documents/6358/254_v5_Anastrazole.pdf
2. **HPRA Anastrozole SmPC cited by NCCP**
   https://assets.hpra.ie/products/Human/20912/Licence_PA0749-028-001_25072025123349.pdf
3. **Pan-Asian adapted ESMO early breast cancer guidelines**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC11145753/
4. **Pan-Asian adapted ESMO metastatic breast cancer guidelines**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC10186487/
5. **NICE NG 101: early and locally advanced breast cancer**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
6. **ESO-ESMO ABC 5 consensus: tumour-marker interpretation**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC7510449/
7. **IOF/CABS/ECTS/IEG/ESCEO/IMS/SIOG updated joint statement on aromatase-inhibitor-associated bone loss**
   https://www.esceo.org/sites/esceo/files/pdf/1-s2.0-S2212137425000351-main.pdf
8. **ATAC: ten-year analysis of anastrozole versus tamoxifen**
   https://pubmed.ncbi.nlm.nih.gov/21087898/
9. **eviQ 3650 v 6: adjuvant whole-breast radiotherapy**
   https://www.eviq.org.au/radiation-oncology/breast/3650-breast-invasive-cancer-adjuvant-whole-breast